# Exemestane · daily hormone therapy

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

> Clinician supportive-care discussion draft. This is not an issued prescription.

## Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

## Medicine-specific support

- Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.
- No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.
- Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.
- The kidney and liver advice for exemestane differs from some other aromatase inhibitors: a routine dose reduction is not specified, but organ problems still need clinical review. Tell the pharmacist about all medicines and supplements because some can make exemestane less effective. Do not increase the dose yourself. Confirm menopause and any separate ovarian-suppression plan.
- NCCP no renal/hepatic dose adjustment, haemodialysis expected no adjustment; linked SmPC caution. Strong CYP3A4 inducer reduces exposure: review, no Irish-source automatic 50 mg dose.

## Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00376 v4 · Monitoring and treatment map